

26STCV14223 26STCV14223

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Case Number: 26STCV14223 Hearing Date: August 31, 2026 Dept: 307

13 TENTATIVE RULING 9:00 a.m., Monday, August 31, 2026

ROSE

ALARCON v. BLUE CROSS OF CALIFORNIA DBA ANTHEM BLUE CROSS, et al. [26STCV14223]

DEFENDANT

LASALLE MEDICAL ASSOCIATES' MOTION TO STRIKE PLAINTIFF'S COMPLAINT

MEET AND

CONFER: OK Counsel

met and conferred telephonically in compliance with Code Civ. Proc. § 430.41 (Aghalarpour Decl., ¶ 2.)

BACKGROUND:

Insurance coverage dispute and negligence claim arising from medical provider's failure to authorize reasonable referrals

TIMELINE:

Early 2025: Plaintiff Rose Alarcon

("Plaintiff") was covered by a managed health care plan contracted by Defendant Blue Cross of California DBA Anthem Blue Cross ("Anthem") pursuant to a Medi-Cal Managed Care Boilerplate Contract with the Department of Health Care Services ("the DHCS Contract").

September 2025: Plaintiff is assigned to

LaSalle Medical Associates as her medical group. LaSalle contracted with Anthem to act as a downstream subcontractor under the DHCS Contract. Thereafter, Plaintiff

experienced jaw pain, and was diagnosed with a temporomandibular joint (“TMJ”) problem. A request was made to LaSalle for a referral to an oral and maxillofacial surgeon (“OMFS”) who could treat her TMJ problem. LaSalle failed to provide referral to a provider who treated TMJ or who was located within the limits required for care.

November 2025: LaSalle gives Plaintiff a referral to a Dr. Punjabi in Redlands, CA which is 280 miles from Plaintiff’s residence and far outside the 45-mile distance and 75-minute drive limits that applied under Plaintiff’s managed care contract. Plaintiff complained to Anthem about LaSalle’s failure to provide her with a referral to an available OMFS, but Anthem failed to investigate and allowed LaSalle to continue with its delays.

Unknown Date: LaSalle provides Plaintiff with a referral to a spine surgeon for an unrelated back condition. That surgeon recommended Plaintiff undergo a “drastic” lumbar fusion. Plaintiff requested a second opinion, which has not been authorized.

5/4/2026: Plaintiff files the Complaint, alleging causes of action for:

1. Breach of Contract
2. Breach of the Implied Covenant of Good Faith and Fair Dealing
3. Promissory Fraud
4. Negligence

6/23/2026: LaSalle files this Motion to Strike, which is followed by Plaintiff’s Opposition (8/18/2026) and LaSalle’s Reply (8/24/2026).

TENTATIVE

RULING: DEFENDANT LASALLE MEDICAL ASSOCIATES’ MOTION TO STRIKE PLAINTIFF’S COMPLAINT is DENIED.

I. MOTION
TO STRIKE

LaSalle moves to strike the allegations related to Plaintiff's request for punitive damages from LaSalle from the Complaint. Plaintiff seeks punitive damages in connection with her second, third, and fourth causes of action in the Complaint. (Compl., ¶¶ 55, 66, 79.) Only the fourth cause of action for negligence is asserted against LaSalle. (Id., ¶¶ 67-79.)

A.

Plaintiff's Fourth Cause Of Action Pleads

Ordinary Negligence, So It Is Not Governed By Code Civ. Proc. § 425.13

LaSalle argues that Plaintiff fails to comply with the requirements of Code Civ. Proc. § 425.13 in connection with her negligence cause of action against LaSalle. (Mot., at p. 8.) Code Civ. Proc. § 425.13 provides that, "[i]n any action for damages arising out of the professional negligence of a health care provider, no claim for punitive damages shall be included in a complaint or other pleading unless the court enters an order allowing an amended pleading that includes a claim for punitive damages to be filed." (Code Civ. Proc., § 425.13, subd. (a).) "A claim arises out of professional negligence 'if the injury that is the basis for the claim was caused by conduct that was directly related to the rendition of professional services.'" (Divino Plastic Surgery, Inc. v. Superior Court (2022) 78 Cal.App.5th 972, 984 (Divino); accord Cooper v. Superior Court (1997) 56 Cal.App.4th 744, 748.) "The legislative history of section 425.13 and various provisions in California codes demonstrate that the procedural requirements described in the statute do not apply to claims against health care service plans," as opposed to health care providers. (Kaiser Foundation Health Plan, Inc. v. Superior Court (2012) 203 Cal.App.4th 696, 715.)

LaSalle argues that Plaintiff's Complaint is governed by Code Civ. Proc. § 425.13 because LaSalle it is a "health care provider" and Plaintiff's allegations of delays in authorizing her medical procedures fall within the scope of "professional negligence" claims. (Mot., at p. 8.) Plaintiff argues that her Complaint is not governed by Code Civ. Proc. § 425.13 because she alleges that LaSalle negligently failed to perform the managed care duties of a health care service plan that it agreed to perform on behalf of Anthem, as opposed to a traditional "medical malpractice" claim against a health care provider. (Opp., at p. 5.) Thus, the relevant inquiry for the Court is: (1) whether LaSalle was acting as a "health care provider" or a "health care service plan," and if the former, (2) whether LaSalle's conduct

amounted to a “professional negligence” claim.

First, the Court finds that LaSalle is a “health care provider” under the meaning of Code Civ. Proc. § 425.13. “The statute defines ‘health care provider’ by reference to several licensing statutes, including persons licensed ‘pursuant to Division 2 ... of the Business and Professions Code,’ and ‘any clinic ... or health facility, licensed pursuant to Division 2 ... of the Health and Safety Code.’” (Divino, supra, 78 Cal.App.5th at p. 981, quoting Code Civ. Proc., § 425.13, subd. (b).) “Article 2.5 of chapter 1 of division 2 of the Health and Safety Code prescribes the licensing requirements for clinics (Health & Saf. Code, § 1221 et seq.) . . . and article 1 of chapter 2 of division 2 of the Health and Safety Code governs licensing of health facilities (Health & Saf. Code, § 1250 et seq.).” (Divino, supra, 78 Cal.App.5th at p. 981.) “[T]he definition in section 425.13, subdivision (b) of ‘health care provider’ should be read broadly to implement its statutory purpose, protecting this type of health care provider, which delivers services to patients, from potentially unfounded punitive damages claims.” (Palmer v. Superior Court (2002) 103 Cal.App.4th 953, 967 (Palmer).)

Here, Plaintiff alleges LaSalle is a “medical group” which provides medical services and also “contracted with Anthem to act as downstream subcontractor under the DHCS Contract with respect to the duties Anthem delegated to it including the duty to comply with the DHCS Contract and all applicable laws and regulations.” (Compl., ¶¶ 5, 11, 16, 36, 68, 70.) The Court takes judicial notice of the fact that LaSalle is a professional medical corporation. (RJN, Exh. 1.) Based on these facts, it can reasonably be inferred that LaSalle is a medical corporation comprised of licensed professionals who provide medical services to patients. “Under Business and Professions Code sections 2406 and 2408, a medical corporation comprised of licensed professionals may render professional services as long as it is in compliance with the Moscone–Knox Professional Corporation Act (Corp. Code, § 13400 et seq.), which requires that only licensed persons render professional services on behalf of the corporation. (Corp. Code, §§ 13405, 13406, subd. (a).)” (Palmer, supra, 103 Cal.App.4th at p. 963.) Thus, “a medical group comprised of licensed medical practitioners, who provide direct medical services to patients...” constitutes a “health care provider” under the meaning of Code Civ. Proc. § 425.13. (Id., at p. 967.) “It does not make any difference that [a plaintiff] prefers to characterize [the medical group] as a “subscriber

agreement administrator,” rather than a health care provider, or that there are administrative or financial aspects to these services.” (Ibid.) Accordingly, here, it does not matter that LaSalle was performing “utilization management” and “authorization” functions given its status as a medical group comprised of licensed medical professionals performing medical services. (Compl., ¶ 11.)

Second, the Court finds that LaSalle’s alleged breaches of duty do not constitute professional negligence. “‘Professional negligence’ is defined in several sections of [MICRA] as ‘a negligent act or omission to act by a health care provider in the rendering of professional services, which act or omission is the proximate cause of a personal injury or wrongful death, provided that such services are within the scope of services for which the provider is licensed and which are not within any restriction imposed by the licensing agency or licensed hospital.’” (Williams v. Superior Court (1994) 30 Cal.App.4th 318, 322.) “In examining the conduct giving rise to the claim, the court must ask whether the acts performed were such as a medical practitioner ordinarily would be expected to perform in his or her capacity as a health care provider.” (Palmer, supra, 103 Cal.App.4th at p. 968.) For example, the Palmer court concluded that injuries arising from utilization review conduct by medical professionals requires clinical judgment that “would arguably arise out of professional negligence.” (Palmer, supra, 103 Cal.App.4th at p. 972.) “[U]tilization review [must] be conducted by medical professionals, and they must carry out these functions by exercising medical judgment and applying clinical standards.” (Ibid.)

Unlike in Palmer, which involved medical necessity evaluations in the course of a medical group’s performance of utilization review, here, Plaintiff is not alleging negligence in the medical judgment of LaSalle’s physicians, but LaSalle’s failure to provide Plaintiff with adequate and timely referrals after medical necessity judgments had been made. (Compl., ¶¶ 31, 33, 71, 75-76.) Plaintiff specifically alleges that LaSalle “did not dispute [her] medical need to see the specialists in a timely fashion to treat her conditions,” but that LaSalle was negligent by delaying or failing to authorize referrals to such specialists because of their inadequate in-network providers. (Id., ¶ 1.) For example, Plaintiff alleges LaSalle authorized OMFS visits with providers who “either did not treat TMJ or were located far outside the limits required for available care.” (Id., ¶ 31.) Plaintiff’s express allegations that LaSalle’s wrongdoing arose out solely of its administrative failures rather than its failure of medical judgment distinguish her Complaint

from Palmer and support a finding of “ordinary negligence” rather than “professional negligence” because LaSalle’s physician’s clinical judgment was not a proximate cause of Plaintiff’s alleged injuries.

Accordingly,

the Court rejects LaSalle’s argument that Plaintiff’s punitive damages claim is governed by Code Civ. Proc. § 425.13.

B.

Plaintiff Has Alleged Sufficient Facts To

Show LaSalle’s Corporate Leaders Acted With The Requisite Malice, Oppression, Or Fraud To Support A Request For Punitive Damages

Second, LaSalle argues that Plaintiff’s

punitive damages allegations are conclusory and not alleged with adequate factual particularity. (Mot., at pp. 12-13.) Punitive damages are recoverable in tort actions where “the defendant has been guilty of oppression, fraud, or malice.” (Civ. Code § 3294, subd. (a).)[1]

California follows a heightened pleading standard for punitive damages— vague and conclusory allegations are insufficient. (G. D. Searle & Co. v.

Superior Ct. (Ct. App. 1975) 49 Cal. App. 3d 22, 29.)²In the case of corporate defendants, “the punitive

damage statute requires proof of malice among corporate leaders: the “officer[s], director[s], or managing agent[s].” (Cruz v. HomeBase (2000)

83 Cal. App. 4th 160, 167 (Cruz), citing Cal. Civ. Code, § 3294, subd. (b);

see *Grieves v. Superior Ct.* (1984) 157 Cal. App. 3d 159,

167 [“a corporate employer may be liable only if the knowledge, authorization, ratification or act was on the part of an officer, director or managing agent of the corporation.”].)

Here, Plaintiff alleges that LaSalle’s

conduct was “intended to cause injury” to Plaintiff and was carried out “with a willful and conscious disregard of the rights of [Plaintiff]” and that LaSalle “subjected [Plaintiff] to cruel and unjust hardship in conscious disregard to [her] rights...” (Compl., ¶ 79.)

Plaintiff further alleges that LaSalle intentionally misrepresented, deceived, or concealed material facts from Plaintiff with the intent to deprive her of her rights or cause injury.” (Ibid.) For example, Plaintiff alleges that

LaSalle “made material representations to [her] regarding its duties to provide

timely access to specialist care, both in-network and out-of-network.” (Id., ¶ 77.) These allegations are not merely conclusory, but they provide the requisite ultimate facts necessary to show malice, oppression, and fraud as defined by Civ. Code § 3294, subd. (a).

Further, Plaintiff has also alleged that LaSalle’s conduct was “authorized, performed, and ratified by [LaSalle’s] officers and managing agents” because the conduct was part of an ongoing corporate practice by LaSalle, and its officers and management-level employees consented to the conduct. (Id., ¶ 79.) For pleading purposes, these factual allegations are sufficient to impute liability for punitive damages onto the corporation itself. (See Cruz, supra, 83 Cal. App. 4th at p. 167.) Assuming the truth of these allegations, Plaintiff demonstrates that she is entitled to punitive damages from LaSalle in connection with her negligence claim.

Therefore, LaSalle’s motion to strike is
DENIED.

II. EVIDENTIARY OBJECTIONS

Plaintiff
objects to the Declaration of Aria A. Aghalarpour and Exhibit 1 to Defendant’s Request for Judicial Notice (nos. 1-2). Plaintiff’s objections are OVERRULED.

III. REQUEST FOR JUDICIAL NOTICE

Defendant
requests judicial notice of the June 21, 2023 Amended and Restated Articles of Incorporation for LaSalle Medical Associates, a California professional corporation, on file in the office of the Secretary of the State of California. (RJN, Exh. 1.) The Court takes judicial notice of the existence of this document pursuant to Evidence Code section 452, subdivision (c).

Defendant LaSalle Medical Associates to serve
notice of ruling. This tentative ruling (“TR”) shall be the order of the Court unless changed at the hearing and shall by this reference be incorporated into the Minute Order. TR emailed to counsel and posted on court’s

website on 8/28/26 at 3:53.

[1] "Malice" may be demonstrated by allegations of conduct "intended by the defendant to cause injury to plaintiff, or despicable conduct that is carried on by the defendant with a willful and conscious disregard for the rights or safety of others." (Id., § 3294, subd. (c)(1).) "Oppression" may be demonstrated by allegations of "despicable conduct that subjects a person to cruel and unjust hardship in conscious disregard of that person's rights." (Id., § 3294, subd. (c)(2).) "Fraud" may be demonstrated by allegations of "intentional misrepresentation, deceit, or concealment of a material fact known to the defendant with the intention on the part of the defendant of thereby depriving a person of property or legal rights or otherwise causing injury." (Id., § 3294, subd. (c)(3).)